

Wellness Medical Centre

DEXA Bone Density Service

17 Harley Street London W1G 9LJ

Tel: 020 7000 1234

Patient: Smith, Jane

DOB: 15/03/1975 ID: 1044892

Scan date: 04 November 2023

Referring: Dr A. Barnes

## DEXA BONE DENSITOMETRY REPORT

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|

**Clinical Indication:** Routine osteoporosis screening. History of low calcium intake.

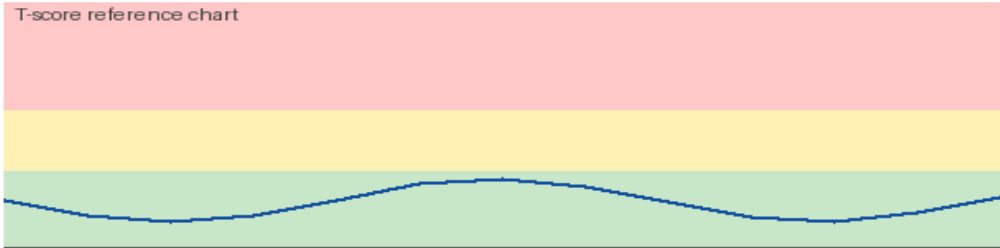
**Equipment:** Hologic Discovery W (S/N 87654)

| Region             | BMD (g/cm²) | T-score | Classification |
|--------------------|-------------|---------|----------------|
| Lumbar Spine L1-L4 | 0.943       | -1.2    | Osteopenia     |
| Left Femoral Neck  | 0.811       | -1.0    | Normal         |
| Left Total Hip     | 0.879       | -0.8    | Normal         |
| Right Femoral Neck | 0.803       | -1.1    | Normal         |
| Right Total Hip    | 0.862       | -0.9    | Normal         |

**CONCLUSION:** Mild osteopenia at lumbar spine (T-score -1.2). All hip measurements within normal range. Recommend calcium supplementation and repeat scan in 2 years.

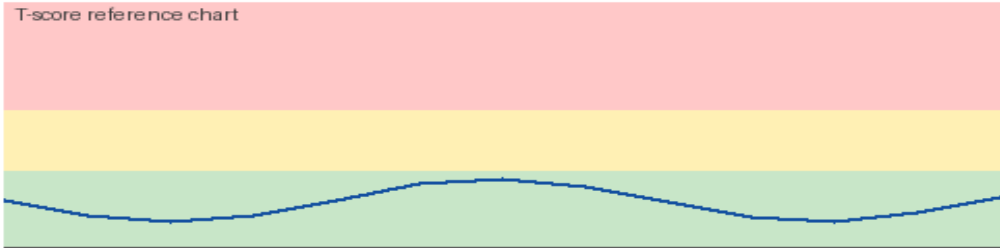
Reported by: Dr C. Patel FRCR, Consultant Radiologist Date: 06 December 2023

# Lumbar Spine L1–L4 — Scan Images



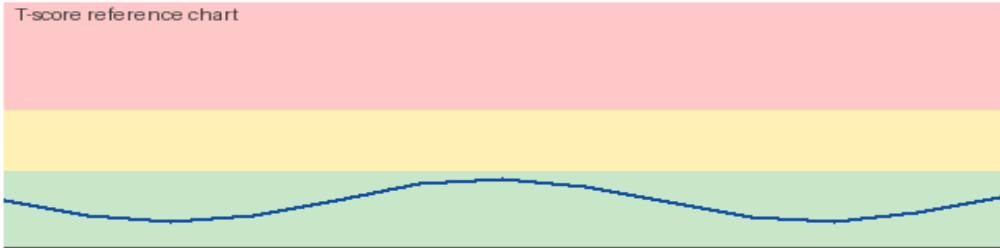
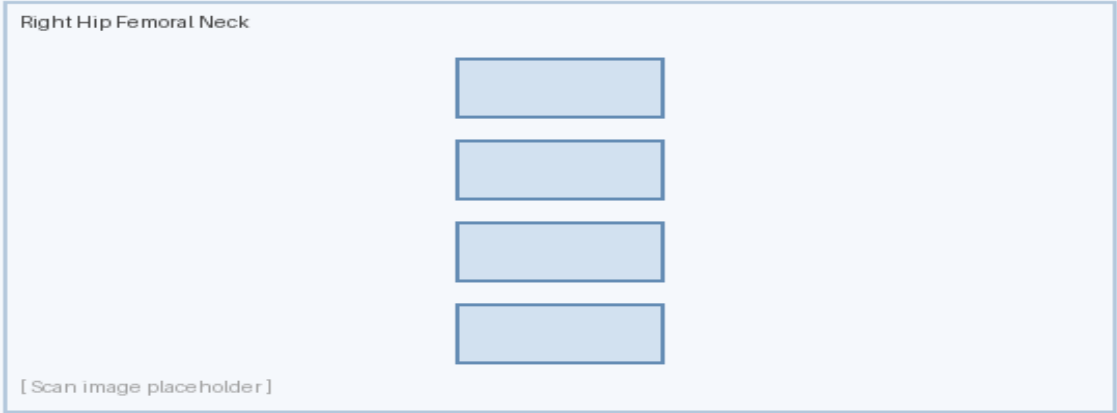
Lumbar spine BMD: 0.943 g/cm<sup>2</sup> | T-score: -1.2 | Z-score: -0.4 | Classification: Osteopenia

Left Hip — Scan Images



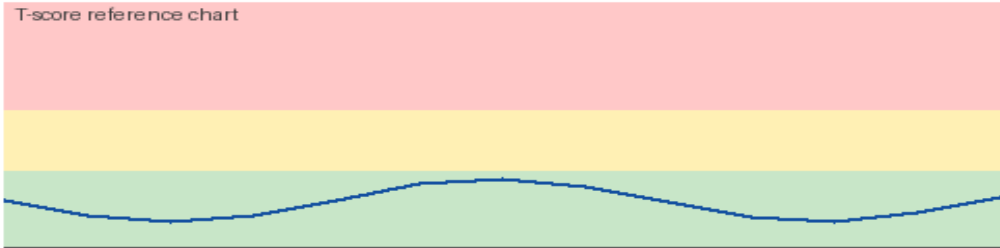
Left femoral neck BMD: 0.811 g/cm<sup>2</sup> | T-score: -1.0 | Z-score: -0.1 | Classification: Normal

# Right Hip — Scan Images



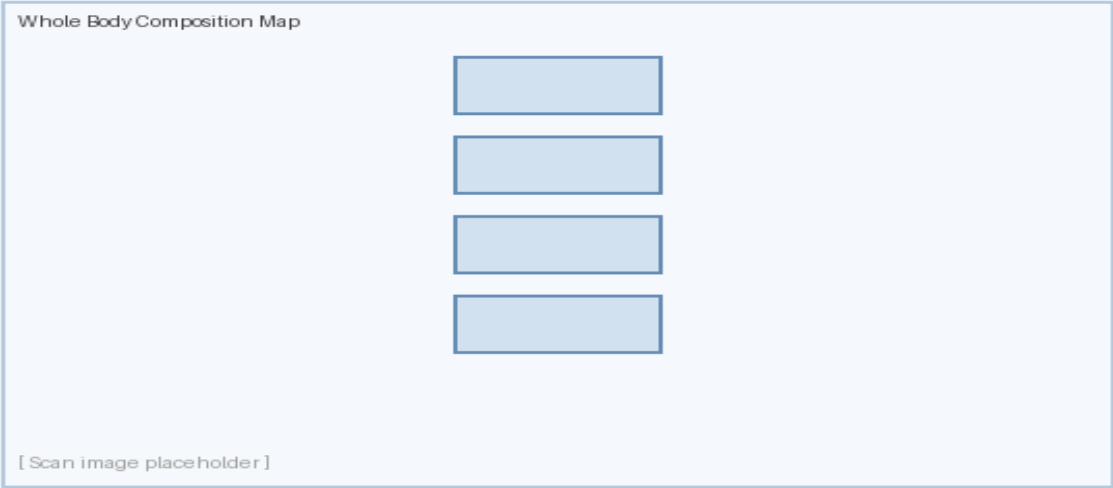
Right femoral neck BMD: 0.803 g/cm<sup>2</sup> | T-score: -1.1 | Z-score: -0.2 | Classification: Normal

Forearm — Scan Images



1/3 radius BMD: 0.661 g/cm<sup>2</sup> | T-score: -0.6 | Z-score: +0.1 | Classification: Normal

# Whole Body Composition



| Region | Fat (%) | Lean (kg) | Total (kg) |
|--------|---------|-----------|------------|
| Arms   | 28.4    | 5.6       | 7.8        |
| Legs   | 31.2    | 14.2      | 20.6       |
| Trunk  | 34.8    | 22.8      | 35.0       |
| Total  | 32.1    | 44.8      | 66.0       |

## Fracture Risk Assessment (FRAX)

Based on clinical risk factors and BMD values, the 10-year fracture probability has been estimated using the FRAX tool (WHO).

| Risk Factor                   | Value    |
|-------------------------------|----------|
| Age                           | 48 years |
| Body weight                   | 64 kg    |
| Height                        | 163 cm   |
| Previous fracture             | No       |
| Parent hip fracture           | No       |
| Current smoker                | No       |
| Glucocorticoids               | No       |
| Rheumatoid arthritis          | No       |
| Secondary osteoporosis        | No       |
| Alcohol ( $\geq 3$ units/day) | No       |

**10-year probability — major osteoporotic fracture: 7.4%** (threshold for treatment: 10%)

**10-year probability — hip fracture: 1.1%** (threshold for treatment: 3%)

Recommendation: No pharmacological intervention indicated at this time. Lifestyle advice given (calcium, vitamin D, weight-bearing exercise). Repeat DEXA in 2 years.



Central London Clinic

12 Portland Place London W1B 1LT

Tel: 020 7000 2200 Fax: 020 7000 2201

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06 December 2023

Re: **Ms Jane Smith** — DOB: 15/03/1975 NHS: 8203456712

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Dear Dr Barnes,

Thank you for referring Ms Jane Smith to our clinic. I reviewed her today and noted the following:

She presents with a 6-month history of fatigue and mild lower back discomfort. Her DEXA scan confirms mild osteopenia at the lumbar spine (T-score -1.2) with normal hip values.

Bloods taken today. I have requested full metabolic panel, 25-OH vitamin D, PTH, and bone turnover markers (P1NP, CTX). Results expected within 5 working days.

I have advised calcium supplementation (1200 mg/day) and vitamin D3 (1000 IU/day). Weight-bearing exercise three times weekly recommended.

I do not feel pharmacological therapy is indicated at present given FRAX probabilities below treatment thresholds. I will review again once blood results are available.

Yours sincerely,

**Dr P. Hamilton** MB BChir MRCP

Consultant Physician — Metabolic Bone Disease

Central London Clinic



Date: 18 October 2023

## MRI LUMBAR SPINE — REPORT

Patient: Ms Jane Smith DOB: 15/03/1975 Ref: L0812537

**Clinical indication:** Lower back pain, osteopenia on DEXA.

**Technique:** 1.5T MRI lumbar spine, sagittal and axial sequences, T1 and T2 weighting.

**Findings:**

- Vertebral body heights and alignment are maintained throughout L1–S1.
- Mild diffuse disc desiccation is noted at L4/5 and L5/S1 without significant disc bulge or protrusion.
- No neural foraminal or central canal stenosis identified.
- Facet joints appear unremarkable with no significant hypertrophy.
- The visualised conus medullaris is at an appropriate level with no intrinsic cord signal change.
- Paraspinal musculature is symmetric.

**Conclusion:**

Mild age-appropriate disc degeneration at L4/5 and L5/S1. No significant nerve root compression. Findings are consistent with early degenerative change and do not explain severe symptoms. Clinical correlation recommended.

Reported by: Dr L. Chen FRCR Consultant Radiologist

## Referral / Consent Form

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LABORATORY REPORT — 30 November 2023

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|

| Test                 | Result | Units                     | Ref Range | Flag |
|----------------------|--------|---------------------------|-----------|------|
| 25-OH Vitamin D      | 42     | nmol/L                    | 50–175    | L    |
| PTH (intact)         | 4.8    | pmol/L                    | 1.6–7.2   |      |
| Calcium (adj)        | 2.38   | mmol/L                    | 2.20–2.60 |      |
| Phosphate            | 1.12   | mmol/L                    | 0.80–1.50 |      |
| Alkaline phosphatase | 72     | U/L                       | 30–130    |      |
| P1NP                 | 48.2   | ng/mL                     | 15–59     |      |
| CTX                  | 0.38   | ng/mL                     | 0.10–0.57 |      |
| eGFR                 | 88     | mL/min/1.73m <sup>2</sup> | ≥60       |      |
| HbA1c                | 38     | mmol/mol                  | <48       |      |
| TSH                  | 2.1    | mU/L                      | 0.4–4.0   |      |

Specimen: 73T166284 Collected: 30 November 2023 Reported: 06 December 2023  
Authorised by: Dr E. Ross MRCPath

LABORATORY REPORT (continued) — Haematology

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

| Test        | Result | Units               | Ref Range | Flag |
|-------------|--------|---------------------|-----------|------|
| Haemoglobin | 128    | g/L                 | 120–160   |      |
| MCV         | 82.4   | fL                  | 80–100    |      |
| MCH         | 27.2   | pg                  | 27–33     | L    |
| Platelets   | 242    | ×10 <sup>9</sup> /L | 150–400   |      |
| White cells | 6.8    | ×10 <sup>9</sup> /L | 4.0–11.0  |      |
| Neutrophils | 4.1    | ×10 <sup>9</sup> /L | 1.8–7.5   |      |
| Lymphocytes | 1.9    | ×10 <sup>9</sup> /L | 1.0–4.0   |      |
| Ferritin    | 18     | µg/L                | 13–150    |      |
| B12         | 342    | ng/L                | 191–663   |      |
| Folate      | 12.4   | µg/L                | ≥4.0      |      |

Specimen: 73T166284 Collected: 30 November 2023 Reported: 06 December 2023

## Insurance / Consent Form

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Date: 22 September 2023

## ENT CONSULTATION LETTER

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

Dear Dr A. Barnes,

Thank you for asking me to see Ms Jane Smith (DOB: 15/03/1975). She presents with a 4-month history of intermittent tinnitus in the left ear and occasional mild vertigo.

### Examination:

- External auditory meati — clear bilaterally
- Tympanic membranes — intact, mobile, normal light reflex
- Rinne test — positive bilaterally
- Weber test — does not lateralise
- Frenzel goggles — no spontaneous or positional nystagmus

### Investigations requested:

Pure-tone audiogram, tympanometry, and vestibular function testing (ENG/VNG).

Given her symptom profile, the most likely aetiology is idiopathic tinnitus +/- early Ménière's disease. I have discussed the natural history and conservative measures (caffeine/sodium restriction, stress management).

Yours sincerely,

Dr T. Nguyen MD FRCS (ORL-HNS)

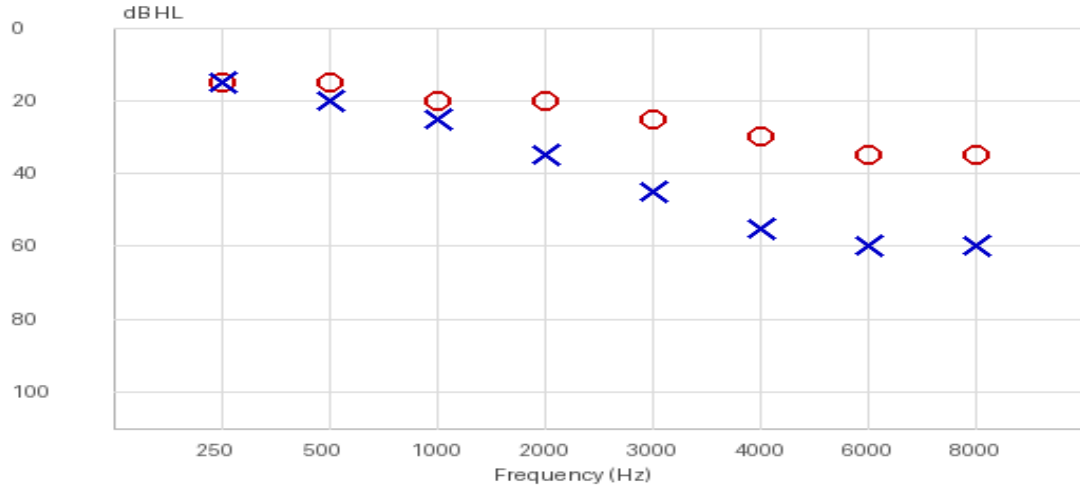
## AUDIOLOGICAL ASSESSMENT

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

### Pure Tone Audiogram



Right ear: normal thresholds ( $\leq 25$  dB HL all frequencies). Left ear: mild low-frequency sensorineural hearing loss (35–60 dB HL 1–8 kHz). Pattern consistent with early endolymphatic hydrops.

Performed by: Dr S. Walsh MSc Audiology Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Saccade Test

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712



Horizontal saccades: latency 190 ms (N), velocity 410°/s (N), accuracy 98% (N).

Reported by: Dr S. Walsh MSc Date: 22 September 2023



## VESTIBULAR FUNCTION TEST — Smooth Pursuit

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712



Gain 0.88 at 0.4 Hz. Mildly reduced (0.72) at 0.8 Hz — borderline low.

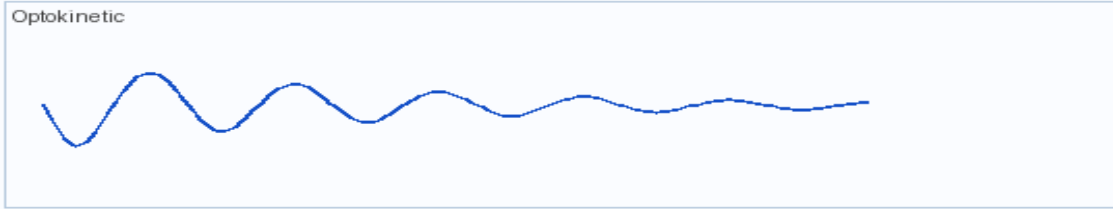
Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Optokinetic

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712



OKN gain symmetric. No directional preponderance.

Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Gaze Nystagmus

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

Gaze Nystagmus



No spontaneous or gaze-evoked nystagmus in primary position or lateral gaze.

Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Positional Nystagmus

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

Positional Nystagmus



No nystagmus on Dix-Hallpike or roll test.

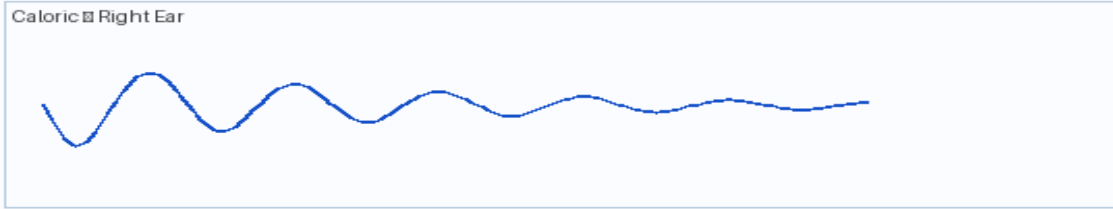
Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Caloric — Right Ear

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712



Right cool: 8°/s. Right warm: 22°/s. Canal paresis: 12% (N <20%).

Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Caloric — Left Ear

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

Caloric Left Ear



Left cool: 14°/s. Left warm: 28°/s. Total response 42°/s vs right 30°/s.

Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Directional Preponderance

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

Directional Preponderance



DP: 17% leftward (borderline; N <20%).

Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — VEMP — Cervical

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712



cVEMP: p13-n23 present bilaterally. Asymmetry ratio 9% (N <35%).

Reported by: Dr S. Walsh MSc Date: 22 September 2023



## VESTIBULAR FUNCTION TEST — VEMP — Ocular

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712



oVEMP: n10 present bilaterally. Asymmetry ratio 11% (N <35%).

Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Head Impulse Test

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

Head Impulse Test



Video HIT: VOR gain 0.92 right, 0.84 left. No corrective saccades.

Reported by: Dr S. Walsh MSc Date: 22 September 2023

## VESTIBULAR FUNCTION TEST — Summary & Impression

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

### Summary & Impression



Mild left-sided caloric reduction and borderline smooth pursuit. Pattern is suggestive of early left peripheral vestibular hypofunction. Recommend vestibular rehabilitation physiotherapy.

Full report authorised by: Dr T. Nguyen FRCS (ORL-HNS) Date: 06 December 2023

Date: 18 October 2023

## ULTRASOUND ABDOMEN — REPORT

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

**Indication:** Routine screen. Family history of gallstones.

**Findings:**

- Liver: normal size and echotexture. No focal lesion identified.
- Gallbladder: contains a 7 mm echogenic focus with posterior acoustic shadowing — gallstone. No wall thickening.
- Common bile duct: 4 mm — within normal limits.
- Pancreas: visualised portions unremarkable.
- Spleen: normal size and echotexture.
- Kidneys: both kidneys normal in size, cortical thickness, and echogenicity.
- No free fluid identified.

**Conclusion:** Single 7 mm gallstone in the gallbladder. No biliary dilatation. Incidental finding — asymptomatic. No intervention required. Annual review suggested.

Reported by: Dr M. Foster FRCR Consultant Radiologist

X-ray / Imaging Request Form

Scanned document

Date: 22 September 2023

## DERMATOLOGY OUTPATIENT LETTER

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Dear Dr A. Barnes,

Thank you for referring Ms Jane Smith. She presented with two pigmented lesions of concern on the right forearm, present for approximately 8 months.

### **Dermoscopic findings:**

Lesion 1 (5 mm, right mid-forearm): regular pigment network, no atypical vessels. ABCDE criteria: asymmetry borderline, border regular, colour uniform tan — probable common acquired naevus.

Lesion 2 (3 mm, right wrist): homogeneous pink-tan pattern, no concerning features. Likely dermatofibroma.

**Plan:** No excision indicated at present. Patient educated in ABCDE self-monitoring. Annual skin surveillance recommended given Fitzpatrick type II and UV exposure history. Broad-spectrum SPF50 sunscreen advised.

Yours sincerely,

Dr L. Chen MRCP (Derm) Consultant Dermatologist

## SPECIALIST REPORTS — SECTION 2

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Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712

Compiled: 06 December 2023

## COLONOSCOPY REPORT

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Date of procedure: 18 October 2023 Endoscopist: Mr D. King FRCS

**Indication:** Bowel cancer screening; family history (maternal uncle — colorectal carcinoma age 62).

### Procedure:

Colonoscopy performed under Entonox sedation. Bowel preparation with Moviprep — adequate (Boston 8/9). Cecum intubated; appendix orifice identified. Procedure time: 28 minutes.

### Findings:

- Rectum: normal mucosa. No polyps.
- Sigmoid colon: normal. Two small (3–4 mm) diminutive polyps removed by cold biopsy forceps — histology pending.
- Descending colon: diverticula present in descending and sigmoid colon. Uncomplicated diverticulosis.
- Transverse colon: normal mucosa.
- Ascending colon and cecum: normal.
- Terminal ileum: intubated 15 cm — normal.

### Conclusion:

Two diminutive sigmoid polyps (likely hyperplastic). Uncomplicated diverticulosis. No evidence of colorectal neoplasia. Repeat colonoscopy in 5 years.

Reported by: Mr D. King FRCS (Gen Surg) OneHealth Medical



## HISTOPATHOLOGY REPORT — Colonic Polyps

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Specimen received: 18 October 2023 Lab ref: 23T109361

**Macroscopic description:**

Two fragments of colonic mucosa received in formalin. Fragment 1: 3 mm. Fragment 2: 4 mm.

**Microscopic description:**

Both fragments show hyperplastic polyp morphology with serrated architecture, no dysplasia, and no evidence of neoplasia.

**Diagnosis: Hyperplastic polyp ×2 (sigmoid). No dysplasia. No malignancy.**

Authorised by: Dr E. Ross FRCPath Consultant Histopathologist 06 December 2023

## ORTHOPAEDIC CONSULTATION

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Date: 18 October 2023 Surgeon: Mr R. Thompson

Thank you for referring Ms Jane Smith. She presents with right knee pain and intermittent swelling for approximately 6 months, worse on stairs.

**Examination:**

- Gait: mildly antalgic on right.
- Range of motion: 0–130° right, 0–140° left.
- McMurray test: equivocal — slight discomfort medial compartment.
- Lachman test: negative.
- Varus/valgus stress: stable.
- Effusion: small right knee effusion palpable.

**Plan:** MRI right knee requested. Physiotherapy initiated (VMO strengthening). NSAID PRN. Review in 8 weeks with imaging.

Yours sincerely,

Mr R. Thompson FRCS (Tr&Orth;)

Atlantic Medical Services

3 Upper Wimpole Street London W1G 6LB

Tel: 020 7000 6600

## MRI KNEE REPORT — Right

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Date: 18 October 2023 Modality: 1.5T MRI Sequences: PD, T1, STIR

- Medial meniscus: grade 2 signal in posterior horn — no definite tear on sagittal sequences.
- Lateral meniscus: normal.
- ACL: intact, normal signal.
- PCL: intact.
- Collateral ligaments: intact bilaterally.
- Cartilage: grade 2 chondral change medial femoral condyle.
- Small joint effusion — no loose bodies.
- Mild pes anserine bursitis.

Reported by: Dr L. Chen FRCR Atlantic Medical Services

Atlantic Medical Services

3 Upper Wimpole Street London W1G 6LB

Tel: 020 7000 6600

## MRI KNEE REPORT — Left (for comparison)

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Date: 18 October 2023 Modality: 1.5T MRI Sequences: PD, T1, STIR

- Menisci: intact.
- Ligaments: intact.
- Cartilage: normal.
- No effusion.

Reported by: Dr L. Chen FRCR Atlantic Medical Services

Atlantic Medical Services

3 Upper Wimpole Street London W1G 6LB

Tel: 020 7000 6600

## ORTHOPAEDIC FOLLOW-UP — 8 WEEKS

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Ms Jane Smith returned for review. MRI confirms grade 2 medial meniscus signal without discrete tear, and grade 2 chondral change medial femoral condyle. Physiotherapy has been beneficial — pain reduced from 6/10 to 3/10 VAS. Effusion resolved. I have advised continuing physiotherapy for a further 12 weeks and weight management. Knee injection not indicated at this stage. No surgical intervention planned.

Yours sincerely,

Mr R. Thompson FRCS (Tr&Orth;)

Atlantic Medical Services

3 Upper Wimpole Street London W1G 6LB

Tel: 020 7000 6600

## ORTHOPAEDIC FOLLOW-UP — 20 WEEKS

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Ms Jane Smith reports substantial improvement. Pain now 1/10 VAS. Full range of motion restored. Functional outcome score (KOOS) 82/100. No further specialist follow-up required — discharge to GP with open access if symptoms recur. Advised to continue home exercise programme.

Yours sincerely,

Mr R. Thompson FRCS (Tr&Orth;)

## BREAST IMAGING REPORT

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Date: 04 November 2023 Radiologist: Dr M. Foster

**Indication:** Routine screening mammogram (age 48, no family history).

**Mammogram (bilateral, CC and MLO views):**

Breast density: B (scattered areas of fibroglandular density). No suspicious masses, calcifications, architectural distortion, or skin changes. Axillary lymph nodes appear unremarkable.

**Breast Ultrasound:**

Targeted ultrasound performed due to palpable focus in left upper outer quadrant. 7 mm simple cyst identified — thin walls, anechoic, posterior acoustic enhancement. BI-RADS 2 — benign finding. No solid or complex lesions.

**Overall assessment: BI-RADS 2 — Benign**

Recommendation: Routine annual mammographic screening.

Reported by: Dr M. Foster FRCR Breast Radiologist

BREAST ULTRASOUND — IMAGE ANNOTATION

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|



7 mm anechoic cyst with thin walls and posterior acoustic enhancement. No internal septations or solid components. BI-RADS 2.

Sonographer: Dr S. Walsh DCR(R) Sonographer Kensington Hospital



TRANSTHORACIC ECHOCARDIOGRAM REPORT

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|

Date: 04 November 2023 Sonographer: Dr S. Walsh Reporting cardiologist: Dr C. Patel

Indication: Palpitations, family history of hypertrophic cardiomyopathy.

2D Measurements:

| Parameter      | Value | Normal   |
|----------------|-------|----------|
| LVEDD          | 47 mm | 42–58 mm |
| LVESD          | 30 mm | 25–40 mm |
| IVSd           | 9 mm  | 6–10 mm  |
| LVPWd          | 9 mm  | 6–10 mm  |
| LA diameter    | 36 mm | 27–38 mm |
| Aortic root    | 30 mm | 20–37 mm |
| LVEF (biplane) | 62%   | >55%     |

Reported by: Dr C. Patel MRCP (Cardiology) Central London Clinic

Central London Clinic

12 Portland Place London W1B 1LT

Tel: 020 7000 2200

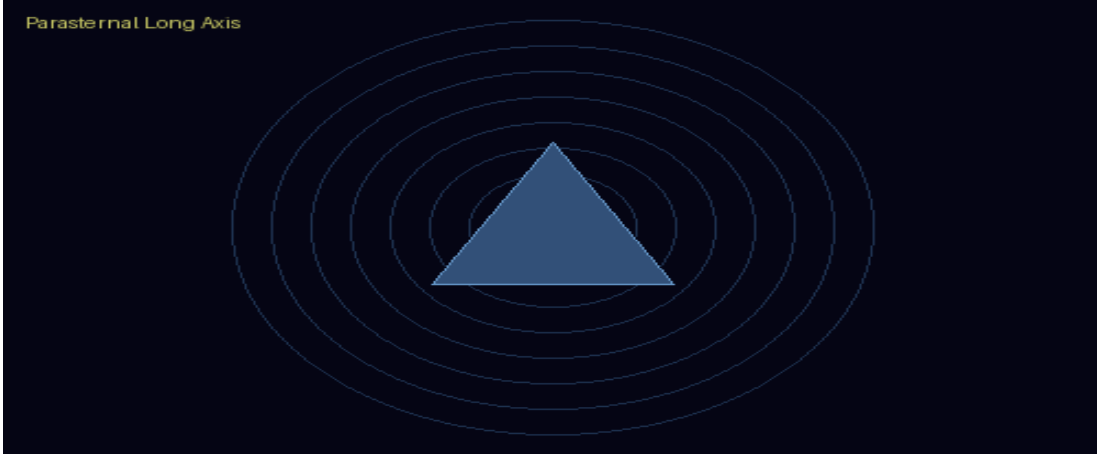
## ECHOCARDIOGRAM — IMAGE VIEWS

Patient: Ms Jane Smith

DOB: 15/03/1975

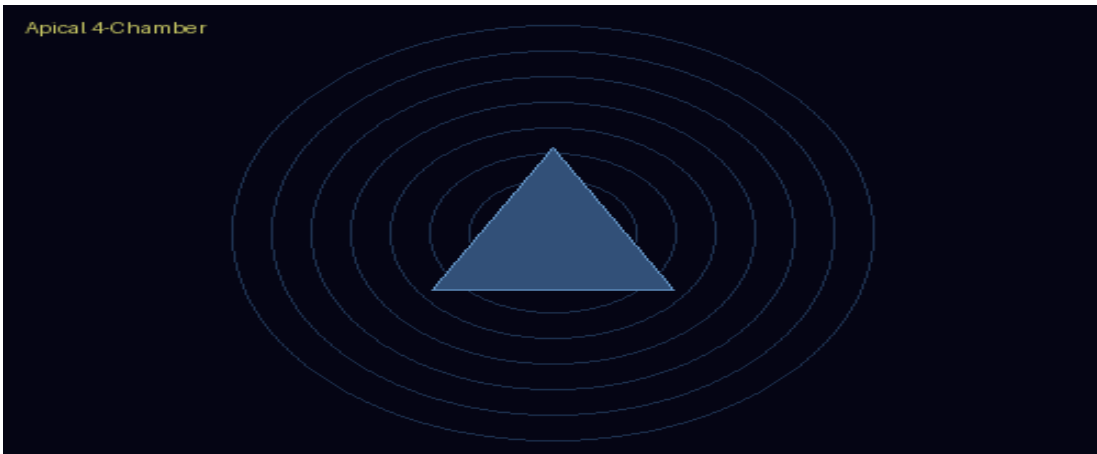
NHS: 8203456712

Parasternal Long Axis



View: Parasternal Long Axis

Apical 4-Chamber



View: Apical 4-Chamber

## ECHOCARDIOGRAM — Doppler Assessment & Conclusions

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

### Doppler findings:

- Mitral valve: normal leaflet morphology. No mitral regurgitation. E/A ratio 1.4 (normal).
- Aortic valve: tricuspid, normal opening. Peak gradient 6 mmHg. No aortic regurgitation.
- Tricuspid valve: trace tricuspid regurgitation — estimated RVSP 26 mmHg (normal).
- Pulmonary valve: normal.

### Conclusion:

Normal left ventricular size and systolic function (EF 62%). No hypertrophy to suggest HCM. No significant valvular disease. Normal diastolic function. No pericardial effusion. Reassuring study — no evidence of structural cardiomyopathy.

Report authorised by: Dr C. Patel MRCP Consultant Cardiologist 06 December 2023

Date: 22 September 2023

## GYNAECOLOGY OUTPATIENT LETTER

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Dear Dr A. Barnes,

Thank you for referring Ms Jane Smith. She attended for review of perimenopausal symptoms: irregular cycles, vasomotor symptoms, and sleep disturbance for approximately 12 months.

FSH 18.2 IU/L, LH 12.4 IU/L, oestradiol 180 pmol/L — consistent with perimenopause. AMH < 0.5 pmol/L.

Pelvic ultrasound: anteverted uterus (7 × 4 × 3 cm), endometrium 6 mm (appropriate). Right ovary 2.8 cm — normal. Left ovary 2.4 cm — small simple follicle (1.2 cm). No free fluid.

I have discussed hormone replacement therapy. Given absence of contraindications, I have commenced transdermal oestradiol (Evorel 50 mcg patch twice weekly) plus Utrogestan 100 mg nocte days 1–12 (sequential HRT). Safety netting and breast awareness advice provided.

Repeat FSH/oestradiol in 3 months. Cervical screen up-to-date.

Yours sincerely,

Dr P. Hamilton MRCOG Consultant Gynaecologist

Blood Test Request Form ■ TDL

Scanned document

Blood Test Request Form - Copy

Scanned document

## SECOND OPINION — MRI LUMBAR SPINE

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Original study date: 18 October 2023 Second opinion requested: 04 November 2023

**Reason for second opinion:**

Persistent back pain despite conservative management. Patient requested specialist neuroradiology review.

**Technique review:**

Images reviewed on PACS. 1.5T, sagittal T1/T2/STIR and axial T2. Image quality: adequate.

**Detailed findings (re-read):**

- L3/4: minimal disc bulge, no significant foraminal encroachment.
- L4/5: disc desiccation confirmed. Mild diffuse posterior disc bulge abutting anterior dural sac — no significant central canal stenosis (AP diameter 12 mm at narrowest). Bilateral subarticular zone narrowing — no exiting root compression.
- L5/S1: mild disc desiccation. No bulge. Mild bilateral foraminal narrowing — no frank stenosis.
- Vertebral end-plates: Modic type 1 change at L4 inferior and L5 superior end-plates — active inflammatory change consistent with discogenic pain source.
- Facet joints: mild L4/5 and L5/S1 facet arthropathy.
- Sacroiliac joints: symmetric, no inflammatory change.

To be continued on next page...

## SECOND OPINION — MRI LUMBAR SPINE (continued)

Patient: Ms Jane Smith

DOB: 15/03/1975

NHS: 8203456712

### Comparison with prior report:

The original report described 'mild diffuse disc desiccation at L4/5 and L5/S1 without significant disc bulge or protrusion.' On re-review, I concur with this characterisation but would additionally highlight the Modic type 1 end-plate changes at L4/5 as a clinically relevant finding that may account for her ongoing pain and was not highlighted in the original report.

### Clinical correlation:

Modic type 1 changes are associated with active discogenic pain and may respond to targeted interventional treatment (intradiscal steroid, bisphosphonates, or anti-inflammatory protocols). Recommend pain medicine consultation.

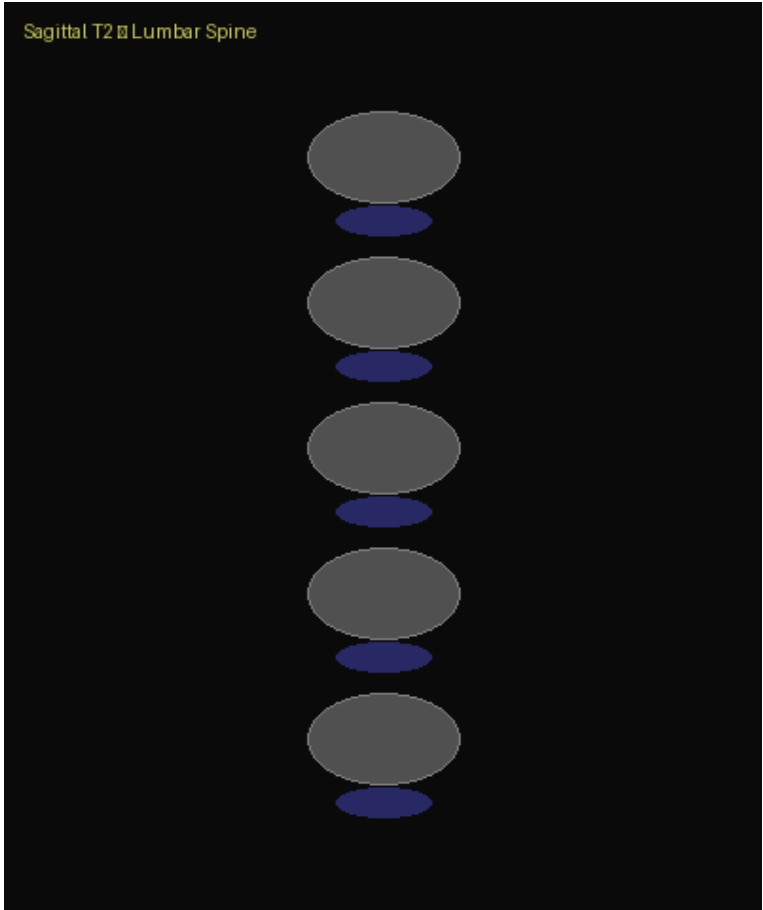
Reported by: Dr P. Hamilton FRCR Neuroradiology

Second opinion review: 06 December 2023



MRI LUMBAR SPINE — Key Images

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|



Sagittal T2: Modic type 1 signal (low T1/high T2) at L4/5 end-plates annotated.

## Pathology / Lab Request Form

Scanned document

LABORATORY REPORT — Hormone & Lipid Panel — 30 November 2023

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|

| Test                | Result | Units  | Ref Range        | Flag |
|---------------------|--------|--------|------------------|------|
| FSH                 | 18.2   | IU/L   | 2–8 (follicular) | H    |
| LH                  | 12.4   | IU/L   | 2–15             |      |
| Oestradiol          | 180    | pmol/L | 150–1500 (foll.) |      |
| Progesterone        | 2.1    | nmol/L | >30 (luteal)     | L    |
| AMH                 | < 0.5  | pmol/L | 1.0–14.5         | L    |
| Testosterone (T)    | 0.8    | nmol/L | 0.3–2.0          |      |
| SHBG                | 62     | nmol/L | 18–114           |      |
| Free androgen index | 1.3    | %      | 0.5–4.3          |      |
| Prolactin           | 280    | mIU/L  | 102–496          |      |
| DHEA-S              | 4.2    | μmol/L | 2.7–9.2          |      |

Specimen: 23T109361 Collected: 30 November 2023 Reported: 06 December 2023

LABORATORY REPORT — Lipid Profile & Metabolic

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|

| Test                | Result | Units    | Ref Range | Flag |
|---------------------|--------|----------|-----------|------|
| Total cholesterol   | 5.2    | mmol/L   | < 5.0     | H    |
| HDL cholesterol     | 1.6    | mmol/L   | > 1.2     |      |
| LDL cholesterol     | 3.1    | mmol/L   | < 3.0     | H    |
| Non-HDL cholesterol | 3.6    | mmol/L   | < 3.8     |      |
| Triglycerides       | 1.1    | mmol/L   | < 1.7     |      |
| TC:HDL ratio        | 3.3    |          | < 4.0     |      |
| HbA1c               | 38     | mmol/mol | < 48      |      |
| Fasting glucose     | 4.9    | mmol/L   | 3.9–6.0   |      |
| hs-CRP              | 1.8    | mg/L     | < 3.0     |      |
| Uric acid           | 280    | μmol/L   | 155–360   |      |

Specimen: 23T109361 Collected: 30 November 2023 Reported: 06 December 2023  
Authorised by: Dr E. Ross MRCPath

## CT CHEST / UPPER ABDOMEN — REPORT

**Patient:** Ms Jane Smith

**DOB:** 15/03/1975

**NHS:** 8203456712

Date: 04 November 2023 Radiologist: Dr L. Chen Modality: 64-slice MDCT, IV contrast

**Indication:** Persistent cough, screen for pulmonary pathology.

### Lungs and airways:

- Lungs fully expanded, no pneumothorax.
- Two sub-centimetre pulmonary nodules: right lower lobe (4 mm) and right middle lobe (3 mm). Below Fleischner threshold — no follow-up required (non-smoker, nodule < 6 mm).
- No consolidation, pleural effusion, or mediastinal lymphadenopathy.
- Airways patent to subsegmental level.

### Upper abdomen (from chest scan):

- Liver: normal enhancement, no focal lesion.
- Gallbladder: previously noted gallstone confirmed (7 mm).
- Spleen, pancreas, adrenals: unremarkable.
- Kidneys: bilateral, symmetric, no hydronephrosis.

**Conclusion:** Incidental sub-centimetre pulmonary nodules — below Fleischner Society threshold, no CT follow-up required. Confirmed gallstone. No other significant pathology.

Reported by: Dr L. Chen FRCR Consultant Radiologist 06 December 2023

CT CHEST — Key Images & Records Compilation Summary

|                        |                 |                 |
|------------------------|-----------------|-----------------|
| Patient: Ms Jane Smith | DOB: 15/03/1975 | NHS: 8203456712 |
|------------------------|-----------------|-----------------|



HEALTH RECORDS COMPILATION — DOCUMENT SUMMARY

| Pages | Document                       | Institution                   | Date              |
|-------|--------------------------------|-------------------------------|-------------------|
| 1–7   | DEXA Bone Densitometry         | Wellness Medical Centre       | 04 November 2023  |
| 8     | Metabolic Bone Disease Letter  | Central London Clinic         | 06 December 2023  |
| 9     | MRI Lumbar Spine Report        | Northgate Private Hospital    | 18 October 2023   |
| 10    | Referral Form (scanned)        | —                             | —                 |
| 11–12 | TDL Lab — Bone/Metabolic       | TDL Pathology                 | 30 November 2023  |
| 13    | Insurance Form (scanned)       | —                             | —                 |
| 14–15 | ENT Consultation + Audiogram   | Wimpole ENT                   | 22 September 2023 |
| 16–27 | Vestibular Function Tests ×12  | Wimpole ENT                   | 22 September 2023 |
| 28    | Ultrasound Abdomen             | London Specialist Diagnostics | 18 October 2023   |
| 29    | Imaging Request (scanned)      | —                             | —                 |
| 30    | Dermatology Letter             | Meridian Clinic               | 22 September 2023 |
| 31    | Section Divider                | —                             | —                 |
| 32–33 | Colonoscopy + Histopathology   | OneHealth Medical             | 18 October 2023   |
| 34–38 | Orthopaedic + MRI Knee ×2 + FU | Atlantic Medical Services     | 18 October 2023   |
| 39–40 | Breast Imaging (Mammo + US)    | Kensington Hospital           | 04 November 2023  |
| 41–43 | Echocardiogram ×3              | Central London Clinic         | 04 November 2023  |
| 44    | Gynaecology Letter             | Kensington Hospital           | 22 September 2023 |
| 45–46 | Lab Request Forms (scanned ×2) | —                             | —                 |
| 47–49 | MRI Second Opinion (Mayo)      | Mayo Clinic Labs              | 06 December 2023  |
| 50    | Pathology Request (scanned)    | —                             | —                 |
| 51–52 | TDL Lab — Hormones & Lipids    | TDL Pathology                 | 30 November 2023  |
| 53–54 | CT Chest/Abdomen + Summary     | London Specialist Diagnostics | 04 November 2023  |